



Exploring clinical practice education for student midwives: Hospital and community opportunities and challenges

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ARTICLE INFO

Keywords:

Health education
Healthcare
Learning environment
Midwifery education
Midwifery
Training support

ABSTRACT

Understanding the experiences of student midwives during clinical training is crucial amid evolving midwifery practices in the Philippines. This training is a vital component of their education, offering real-world experience in hospital and community settings. Although existing literature covers various aspects of midwifery training, detailed accounts of the specific challenges and opportunities faced by Filipino student midwives are limited. This study explores the experiences of student midwives across different levels of training in public and private institutions, focusing on the opportunities and challenges they encountered. A qualitative descriptive research design was conducted with 26 purposively selected student midwives. Data collected through in-depth interviews were analyzed to identify recurring themes. Key themes included the importance of a supportive learning environment, exposure to diverse cases, and the acquisition of practical skills. Challenges such as hierarchical barriers and communication challenges, role confusion, resource limitations and adaptation struggles, balancing autonomy and supervision, emotional toll of challenging situations and limited community experiences were highlighted. The findings stress the need for a more supportive and resourceful training environment. Addressing these challenges through targeted interventions could enhance the learning experience and better prepare student midwives for their professional roles. Collaborative efforts with other healthcare professionals and continuous learning opportunities are also essential for their development. This study offers valuable insights for educators and healthcare institutions to optimize midwifery training programs, ultimately fostering the holistic growth of future midwives.

Introduction

In recent years, the field of midwifery education and practice has undergone significant transformations, marked by historical developments and legislative changes that have shaped the landscape of maternal and newborn care in the Philippines (Canila and Hipolito, 2018). Despite the depth of this existing body of work, a conspicuous gap remains regarding the firsthand experiences of student midwives during their clinical practicum education. Consequently, this study aims to address this gap by delving into the opportunities and challenges encountered by student midwives as they undergo training in hospital and community settings. Through a qualitative descriptive research approach, this study seeks to explore the lived experiences of student midwives, offering a comprehensive understanding of their encounters, perceptions, and insights during their clinical practicum. By engaging in in-depth interviews and analysis, the study aims to uncover various aspects of their training, shedding light on the intricacies of midwifery education and practice in the contemporary Philippine context.

The literature encompassing midwifery education and practice has delved into multifaceted aspects of healthcare discipline. Prior research has undertaken a thorough exploration of the historical underpinnings of midwifery in the Philippines, tracing its emergence to influential figures such as Dr. Jose Fabella, whose visionary contributions paved the way for the establishment of midwifery schools and the formalization of midwifery as a distinct profession (Roces, 2017). These early roots have been foundational in shaping the trajectory of midwifery education and practice within the country. Integral to the evolution of midwifery are legislative milestones that have significantly impacted its autonomy and specialization within the broader healthcare framework. The enactment of Republic Act No. 2382 in 1959 stands as a defining moment that separated midwifery from the domain of medicine, establishing it as an autonomous profession with its own set of regulations and responsibilities (Republic Act [RA] No. 2382, 1959). This legislative measure marked a critical turning point, granting midwifery a distinct identity and underscoring its importance in maternal and newborn care. The repercussions of this legal action reverberate through the

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<https://doi.org/10.1016/j.midw.2024.104186>

Received 2 October 2023; Received in revised form 4 September 2024; Accepted 9 September 2024

Available online 10 September 2024

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present-day landscape of midwifery, shaping the scope and significance of midwives' contributions to healthcare delivery. Furthermore, the recent prominence of House Bill 3882 signals an ongoing commitment to fortifying midwifery education to align with the evolving demands and standards of the healthcare sector (Sineng, 2022).

Moreover, the recent issuance of Commission on Higher Education (CHED) Memorandum Order No. 3, Series of 2023, marks a significant stride in the ongoing advancement of midwifery education in the Philippines (Commission on Higher, 2023). This update heralds a transformative shift in the midwifery curriculum. In contrast to the previous curriculum (CMO No. 33, 2007), which allowed students to undertake the board exam following a two-year midwifery diploma course and subsequently enroll in the Bachelor of Science in Midwifery (BSM) program, the new directive mandates the completion of the full four-year BSM program before being eligible to sit for the board exam. This directive underscores the proactive commitment of the regulatory authority to bolster the educational landscape for aspiring midwives. By meticulously addressing the ever-evolving intricacies of maternal and newborn care, CMO No. 3 (2023) aims to ensure that prospective midwives are not only equipped with essential knowledge, skills, and competencies but also prepared to excel in various future roles, including midwifery entrepreneurship. The enhancement of entrepreneurship courses prepares them to establish their own birthing facilities and enterprises in the future.

Considering these significant advancements, it's crucial to delve into the discernible implications that have surfaced within the financial landscape, particularly concerning affiliation fees and related expenses. One notable development is the convergence of fees between the Bachelor of Science in Midwifery (BSM) program, as outlined in CMO No. 3 of 2023, and the Bachelor of Science in Nursing (BSN) program, as per CMO No. 15 (2017). This convergence naturally sparks an inclination to compare the financial investments associated with both educational paths. It's important to recognize the historical context surrounding the BSM program. Initially structured as a diploma course, it served as a ladderized program for some nursing courses. Consequently, there has been a pervasive connotation that midwifery is subordinate to nursing, leading to discrepancies in salary offerings. This perception has perpetuated the notion that midwives are accorded lower status and remuneration compared to nurses. However, with the transition of the BSM program into a four-year curriculum, it now stands on equal footing with the BSN program in terms of educational investment. This shift in educational structure brings to light the issue of parity in salary considerations. While the cost of education for midwives has aligned with that of nurses, the prevailing disparities in salary levels persist.

Furthermore, a compelling area of exploration revolves around the interplay between midwifery and nursing, particularly within the context of roles in a hospital setting. While distinct roles are evident within the delivery room, the ward introduces a scenario where responsibilities occasionally intermingle, most especially with the advances in midwifery practice. Consequently, student midwives often grapple with the task of defining their distinct roles in this dynamic and collaborative environment. Questions may arise: Who assumes responsibility for the less pleasant tasks and hourly monitoring? Can they contribute to the medical chart or are their entries confined to the TPR sheets only? Who administers medications, and who is qualified to insert IV fluids? These may be some of the questions that encapsulate the multifaceted challenges that student midwives navigate as they strive to define their roles within this multifarious setting. Venturing beyond the confines of healthcare institutions, the community setting offers a more distinct delineation of midwifery roles. These roles are meticulously outlined, further illuminating the comprehensive spectrum of midwifery practice, albeit with occasional intersections. Thus, comprehending the intricacies of midwives' responsibilities in patient care, especially when assigned to ward duties, becomes imperative early in their educational journey.

Adapting to evolving policies and navigating the complexities of roles within hospital and community settings provides a glimpse into the multifaceted challenges and opportunities encountered by student midwives. Recognizing the intricate nature of these dynamics, the researcher undertakes the task of exploring this terrain. The study aims to identify the diverse array of opportunities and challenges faced by student midwives, all through the lens of their unique individual experiences. In particular, the current research addresses two fundamental questions: (1) What are the opportunities that student midwives encounter in both hospital and community settings? (2) What challenges do they experience in these contexts? These inquiries delve into the core of student midwives' experiences, shedding light on the strengths they gain and the hurdles they overcome while traversing the intricate landscape of their education and clinical practice.

Methodology

Research design

This study employs a qualitative descriptive research design, as outlined by Sandelowski (2010), to explore the straightforward experiences and perspectives of student midwives. The focus is on understanding their perceptions of the challenges and opportunities encountered during clinical education. A qualitative descriptive approach is particularly appropriate for this study because it acknowledges the subjective nature of the topic and the diverse experiences of participants (Bradshaw et al., 2017). This approach is especially relevant in healthcare research, where subjective experiences are often central to understanding complex issues (Doyle, 2020). The qualitative descriptive method facilitates understanding of the educational experiences of student midwives, capturing the intricacies of their learning journey. To ensure methodological rigor and transparency, the study adhered to the Consolidated criteria for REporting Qualitative research (COREQ) Checklist (Tong et al., 2007). The COREQ Checklist provides a comprehensive framework for reporting qualitative research, ensuring that all aspects of the study—from design to data collection, analysis, and reporting—meet high standards of quality and accountability.

Participants

The research participants were student midwives enrolled in midwifery education programs at selected institutions in the Bicol region (Region V) of the Philippines, including both public and private institutions. A homogeneity sampling technique was employed to focus on a specific subgroup, reduce variation, and simplify analysis (Palinkas, 2015). The sample comprised first-year ($n = 8$), second-year ($n = 10$), third-year ($n = 4$), and fourth-year ($n = 4$) student midwives. Among them, four were from a private institution, while the rest were from a public university.

The researcher, who also served as the Clinical Instructor for the third-year students, conducted the interviews. The researcher is a male registered nurse, licensed professional teacher, and holds a Master of Arts in Nursing, a Master of Arts in Science Education, and a Doctor of Philosophy in Science Education with a major in Biology. He is an assistant professor with five years of experience in clinical and community health and seven years in teaching research and science. Participants were informed about the research purpose and assured that their honest responses were crucial for improving educational practices. They were made aware that their grades would not be affected by their participation. This transparency aimed to ensure that participants felt comfortable and understood the study's objective to enhance future learning experiences.

Research instruments

The research instrument employed for data collection was

unstructured interviews, allowing participants to share their experiences, perceptions, and insights in an open and flexible manner. Unstructured interviews provided the freedom for participants to elaborate on their thoughts and feelings, enabling a comprehensive exploration of their lived experiences (Mueller and Segal, 2014). Examples of open-ended questions posed during the unstructured interviews include: (1) *Pakilarawan ang isang karanasan sa hospital o community na may malaking epekto sa practice as student midwife?* (Could you describe a particular experience you had in a hospital or community setting that greatly influenced your practice as a student midwife?) (2) *Paano mo nakikita ang papel ng isang student midwife sa loob ng ospital at komunidad?* (How do you see the role of a student midwife in both hospital and community settings?) (3) *Ano ang mga hamon na naranasan mo sa iyong clinical practicum, at paano mo ito nalampasan?* (What challenges did you face during your clinical practicum, and how did you handle or overcome them?) Anu ano ang mga oportunidad na nangyari saiyong habang nagduduty kayo sa ospital at community? (What opportunities have occurred to you while on duty at the hospital and in the community?) These open-ended questions serve as examples and are not strictly followed, allowing the interview to remain unstructured. They are used to prompt participants to share their individual stories and insights, facilitating the collection of rich qualitative data that encapsulates the depth and nuances of their experiences. Subsequent and follow up questions are contingent upon the responses received from participants.

Ethics

The study, proposed at the university level, received prior approval for its methodology in accordance with ethical standards. Before starting the interviews, a total of 26 participants were presented with comprehensive information detailing the study's objectives and their roles as participants. Every effort was made to ensure participants' understanding of the study's purpose, and their informed consent was collected in written form, thus reinforcing the voluntary nature of their participation. Ethical considerations were carefully upheld, encompassing the vital principles of anonymity and confidentiality throughout the entire data collection process. These safeguards were thoughtfully implemented to safeguard the rights and privacy of all participants.

Data gathering procedures

Individual unstructured face-to-face interviews were conducted with each participant to delve into their unique experiences, thoughts, and perceptions regarding their clinical education journey. Field notes were taken during the interviews, and audio recordings were made only with participants' permission. Guided by open-ended questions, these interviews encouraged participants to share detailed narratives, providing rich qualitative data. The interviews, lasting approximately one to two hours per participant, were conducted in private settings within the school, hospital, and community environments during or after their duty hours. These settings were chosen to foster an atmosphere of comfort and openness. To ensure a comprehensive exploration of themes, a follow-up interview was conducted with eight participants, two from each level, who were identified as having particularly rich cases. The selection of these participants was made by the researcher in collaboration with two colleagues—one an expert in education and the other in midwifery practice. Together, they determined which initial interviews contained the most valuable insights for further investigation. Thematic analysis, based on Braun and Clarke's (2006) framework, was used to analyze the data, while member checking enhanced the trustworthiness of the findings by validating participant responses.

Data analysis

The ensuing phase involved the transcription of the recorded interviews verbatim. A thematic analysis approach was adopted, aimed at

identifying recurring patterns, themes, and insights interwoven within participants' accounts. This comprehensive analytical process encompassed coding, categorization, and interpretation of the compiled data following the methods proposed by Creswell and Cresswell (2023) and Braun and Clarke's (2006). Data coding was conducted by three individuals: one expert in education, one in midwifery practice, and the researcher. The coding process involved a detailed description of the coding tree, which was designed to organize and categorize the data systematically. Themes were derived inductively from the data rather than being predetermined. Participant quotations were used to illustrate the identified themes, with each quotation clearly identified by participant number. There was consistency between the data presented and the findings, with major themes clearly delineated. In a bid to enhance the precision and validity of the outcomes, participants were accorded the opportunity to review and endorse the preliminary analysis of their interviews—a practice known as member checking. This participatory mechanism facilitated participants in offering feedback and corroborating the interpretation of their experiences, lending further authenticity to the study's conclusions.

Findings

The responses of the participants were coded and grouped into themes that captured the positive aspects experienced by student midwives during their clinical training in hospital and community settings. Four main themes generated from the analysis, highlighting the positive aspects encountered by student midwives in hospital and community settings: **supportive learning environment, diverse clinical experiences, hands-on learning and skills development, and exposure to real-world challenges** (Fig. 1). This answers the first objective of the study.

Opportunities that student midwives encountered in clinical practicum

Supportive learning environment

Participants in the study consistently conveyed a prevailing sense of support and guidance, underscoring the significance of these attributes in both hospital and community settings. This facet of their experiences was particularly evident in their interactions with more experienced practitioners. Student midwife 11 (second year) illuminated this sentiment by reflecting, *"Yung mga nurses and midwives naman sa hospital ay willing lagi magshare ng alam nila sa amin. Matiyaga silang magexplain ng mga procedures at sumagot sa mga tanong po namin."* (In the hospital, the nurses and senior midwives are always willing to share their knowledge with us. They patiently explain procedures and answer our questions.) This underscores the collaborative and nurturing atmosphere within hospital settings, where experienced professionals willingly engage in mentorship and knowledge transmission. Echoing this sentiment, student midwife 4 (third year) articulated her appreciation for the mentorship she received in the community, stating, *"Nakakatuwa po kung paano yung mga matagal ng midwife sa community ay tinuturuan kami tungkol sa nakasanayan at kultura ng barangay."* (I appreciate how the experienced midwives in the community take the time to teach us about local practices and cultural sensitivities.) This further underscores the role of experienced midwives in community settings as educators, bridging the gap between theoretical knowledge and the practical nuances of local practices. Their willingness to impart valuable insights speaks to the cohesion and mutual respect present within these health-care environments.

Diverse clinical experiences

Apart from the supportive learning environment, student midwives also found value in the diverse range of clinical experiences offered in both settings. Student midwife 2 (fourth year) provided insight into this aspect, stating, *"Na expose ako sa ospital sa iba't-ibang cases tulad ng high-risk pregnancies na mas ok kasi masnatututo pa ako."* (In the hospital, I've

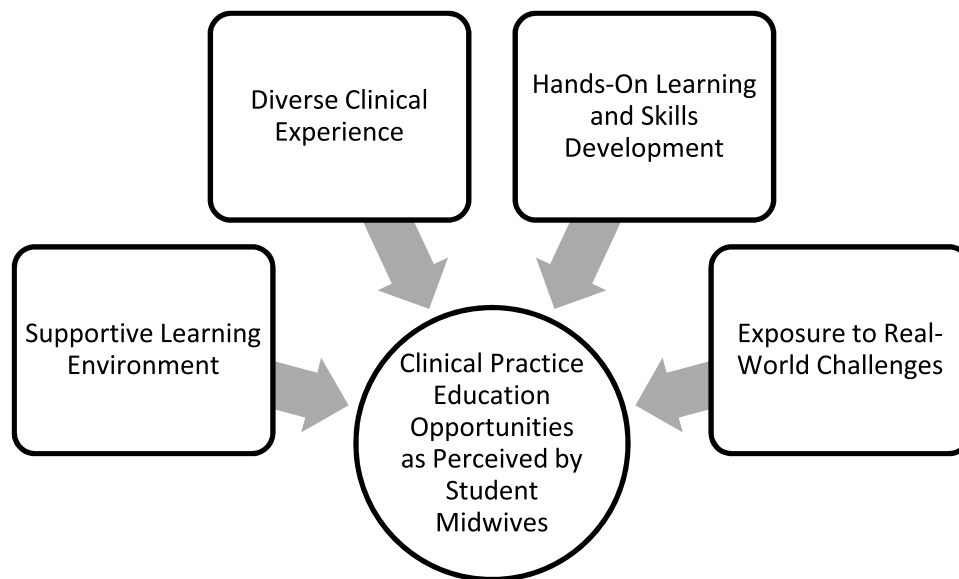


Fig. 1. Clinical practice education opportunities as perceived by student midwives.

been exposed to various complicated cases like high-risk pregnancies, which has expanded my knowledge and skills.) This diversity in case exposure aligns with educational research emphasizing the importance of varied clinical experiences in cultivating well-rounded and adaptable healthcare practitioners. Similarly, student midwife 23 (fourth year) highlighted another dimension of diversity by noting, "*Nakita ko po sa community na importante ang home-based care at personal support para sa mga buntis.*" (Working in the community allowed me to see the importance of home-based care and personalized support for expecting mothers.) These perspectives not only enrich student midwives' understanding of care delivery but also exposes them to the intricacies of providing healthcare within different contexts, including the intimate settings of patients' homes.

Hands-on learning and skills development

Moreover, participants emphasized the opportunity for hands-on learning and skill development, which forms a cornerstone of their education. Student midwife 15 (second year) underscored this by expressing, "*Sa ospital, natutunan ko po ang pagsusuri ng cervixes at fetal monitoring. Yung mga skills na ito ay importante at kailangan namin in the future.*" (In the hospital, I've learned to perform procedures like cervical checks and fetal monitoring. These practical skills are crucial for our future careers.) This reaffirms the practical emphasis of clinical education, equipping student midwives with the competencies required for effective clinical practice. Student midwife 6 (first year) further emphasized the unique aspects of skill development in community settings, stating, "*Sa community, natututunan namin ang pangangalaga at pagbuo ng magandang relasyon sa mga pamilyang naka-assign sa amin.*" (Community settings let us practice holistic care and focus on building relationships with the families we assist). Here, the emphasis shifts towards a more comprehensive and patient-centered approach, underscoring the value of nurturing interpersonal relationships and understanding the broader context of patient care. The merging of practical skills, compassion, and patient interaction serves as a holistic framework for students' growth, ensuring that they emerge as well-prepared and empathetic midwives capable of providing comprehensive care within the dynamic healthcare landscape.

Exposure to real-world challenges

Lastly, participants conveyed their recognition of the educational value inherent in exposure to real-world challenges. Student midwife 1 (first year) elucidated this, noting, "*Sa ospital, may mga sitwasyon na*

kailangan po naming mag-isip ng mabilis. Yung mga ganito po, dinidevelop sa amin ang critical thinking at problem-solving skills." (In the hospital, we face urgent situations that require quick decision-making. This exposure helps us develop our critical thinking and problem-solving skills). This experience of high-pressure scenarios provides an invaluable opportunity for student midwives to cultivate crucial cognitive skills essential for their roles. Furthermore, student midwife 8 (first year) accentuated the adaptability nurtured by community settings, stating, "*Sa community, narealize ko ang kahalagahan ng pagiging resourceful at paghahanap ng mga creative na solusyon.*" (The community setting showed me the importance of adapting to resource limitations and finding creative solutions). This highlights the formative influence of overcoming constraints, fostering a mindset of innovation and resourcefulness—a fundamental attribute in the rapidly evolving landscape of healthcare. In addition, this underlines the necessity for students to familiarize themselves with high-pressure scenarios to prepare them for their future roles.

In addition to the positive aspects encountered, the participants encountered challenges during their clinical practicum in both hospital and community settings. These challenges have led to the identification of six prominent themes generated from their experiences. These themes encompass **hierarchical barriers and communication challenges, role confusion, resource limitations and adaptation struggles, balancing autonomy and supervision, the emotional toll of challenging situations, and limited community experience.**

Challenges that student midwives encountered in clinical practicum

Hierarchical barriers and communication challenges

Navigating the hierarchical structure within hospital settings proved to be a significant challenge for the participants. Student midwife 13 (second year) recounted, "*Minsan po sa ospital ay nakakatakot lapitan ang mga doktor o mga senior staff para magtanong o humingi ng tulong.*" (In the hospital, sometimes it's intimidating to approach doctors or senior staff to ask questions or seek guidance). This sentiment was echoed by student midwife 10 (second year), who emphasized that the hierarchical environment occasionally hindered open communication and hindered their ability to seek clarification or assistance. This theme suggests that despite the supportive atmosphere, certain power dynamics within hospital settings can inhibit effective communication, possibly impacting the comprehensive learning experience of student midwives. These insights highlight the delicate balance between maintaining respect for the established hierarchy and fostering open communication in hospital

settings. While the hospital environment can at times be demanding, this challenge also provides an opportunity for students to cultivate patience and adaptability, crucial attributes in their journey toward becoming skilled and empathetic healthcare professionals.

Role confusion

In the ward setting, where both nurses and midwives undergo a four-year course, the delineation of specific roles poses a persistent challenge. Student midwife 12 (second year) expressed, "*Sa ward, minsan hindi malinaw kung sino talaga ang dapat na may responsibilidad sa anumang gawain. Gusto ko rin sana yung ginagawa ng nurses, pero di ko alam kung pwede ba.*" (In the ward, sometimes it's not clear who should really be responsible for any task. I would also like to do what nurses are doing, but I'm not sure if I am allowed.) This sentiment was echoed by student midwife 6 (first year), who highlighted instances of role ambiguity leading to inefficiencies and confusion in patient care delivery. Despite their equivalent educational investment, midwives often find themselves operating under the direction of nurses in the ward, raising questions about the clarity and fairness of role distribution. This underscores the pressing need for greater clarity and communication regarding the distinct roles and responsibilities of nurses and midwives, especially within the ward context, ensuring equitable opportunities for professional growth and optimal patient care outcomes.

Resource limitations and adaptation struggles

Student midwives' experiences in the community settings revealed challenges arising from resource limitations. Student midwife 21 (second year) highlighted, "*Sa community po talaga, madalas kulang ang mga gamit. Kailangan po talagang maging creative para makapagbigay ng quality care.*" (In the community, we often encounter shortages of supplies and equipment. We need to get creative to provide quality care). Similarly, student midwife 12 (second year) expressed frustration, stating, "*Ang pag-adapt sa mga kakulangan at saka po yung paghanap ng mga mapagkukunan ng mga gamit pag kulang po sa community ay mahirap po talaga. Kailangan naming gamitin ang mga bagay na available, na hindi palaging ideal.*" (Adapting to the limited resources in the community can be challenging. We need to make do with what's available, which is not always ideal). These responses underscore the resource constraints inherent in community healthcare contexts and highlight the need for student midwives to develop adaptive strategies in the face of scarcity. Additionally, these highlight the vital role of ingenuity and adaptability in community healthcare, as well as the importance of preparing student midwives to tackle challenges arising from limited resources.

Student midwife 12's additional comment highlights another significant challenge: the financial burden associated with acquiring necessary supplies. She noted, "*Ang hirap din po kasi kami po ang bumibili ng mga gagamitin like gloves, etc. Eh kulang na rin po yung budget naming, lalo na po mahal na rin ang RLE fee*" (It's also difficult because we are the ones buying supplies like gloves, etc. Our budget is also insufficient, especially since the RLE fee has become expensive). This statement points to the financial constraints faced by student midwives, which further complicate their ability to provide adequate care in community settings. Student midwife 25's (first year) comment, "*Minsan nga po sir hindi na kami makaduty kasi wala rin kaming pamasaha*" (Sometimes we can not even report for duty because we do not have fare money), further underscores the financial struggles that impact their ability to fulfill their responsibilities. This issue not only affects their participation in practical training but also limits their overall learning experience and engagement in community healthcare.

Balancing autonomy and supervision

While participants appreciated the hands-on experiences offered in both settings, they also grappled with finding the right balance between autonomy and supervision. Student midwife 13 (second year) reflected, "*Minsan gusto ko sana na mas nakakadecide ako ng sarili ko kaya lang naiinitindihan ko rin ang kahalagahan ng pagsupervise sa amin para sa ok*

din ang pasyente." (Sometimes, I wish I had more independence to make decisions, but I also understand the importance of being supervised for patient safety). This sentiment was echoed by student midwife 14 (third year), who added, "*Medyo challenging din ang pagbalanse ng pagpraktis ko ng skills sa sarili ko at paggawa na may guidance. Gusto ko rin kasi na prepared ako in the future, pero gusto ko rin malaman kung tama ba talaga ang ginagawa ko.*" (Finding that middle ground between practicing skills on my own and having guidance can be challenging. I want to be prepared, but I also want to ensure I'm doing things correctly). These perspectives reveal the delicate equilibrium student midwives strive to maintain between gaining practical experience and the need for supervision to ensure optimal patient care. These insights highlight the intricate navigation of the learning process, as student midwives work to integrate autonomy, guidance, and patient well-being, all while being mindful of the responsibilities and accountability that come with healthcare practice.

Emotional toll of challenging situations

Participants acknowledged the emotional toll that challenging situations could exact on them. Student midwife 25 (first year) shared, "*sa ospital po sir, minsan di ko na kinakaya emotionally kapag may mga complications o kaya yung grabe na ang sakit ng patients. Importante po talaga as student midwives na marunong kami magcope sa mga ganitong sitwasyon para di rin kami maapektuhan at mabigay naming ang best naming sa pag-alaga ng mga pasyente.*" (In the hospital, witnessing complications or difficult cases can be emotionally draining. It's important to find ways to cope with these emotions while continuing to provide care). This insight highlights the necessity for student midwives to develop coping mechanisms to navigate emotionally demanding scenarios, safeguarding their own well-being while delivering patient-centered care. The balance between empathy and professionalism is crucial in the field of healthcare (Bas-Sarmiento et al., 2020). Students need to learn how to manage their emotions effectively so that they can provide the best care possible to their patients while also maintaining their own well-being.

Limited community experience

While community settings offer unique opportunities, some participants expressed concerns about the lack of diversity in case presentations. Student midwife 16 (second year) stated, "*Sa community po, madalas pare pareho lang po yung ginagawa, kaya minsan nagworry po ako na hindi namin maexperience yung ibang sakit o kaya naman cases po.*" (In the community, we often see similar cases, and I worry that I might miss out on exposure to a wider range of conditions and complications). This concern touches on the need for comprehensive exposure to various scenarios to ensure that student midwives are well-prepared for the spectrum of challenges they may face in their careers. Student midwife 21 (second year) also contributed their perspective, acknowledging, "*Madalas pare-pareho naman tlaga ang ginagawa sa community, pero yung pagkakaiba niya sa hospital ay marami kang makakahalubilo na iba't ibang tao at pamilya na meron ding iba-ibang health practices.*" (Often, the tasks in the community are quite similar, but what sets it apart from the hospital is that you get to interact with a wide variety of people and families, each with their own unique health practices.) This viewpoint emphasizes that even within repetitive routines, community settings offer invaluable insight into the varying perspectives, lifestyles, and cultural norms that shape health practices, enriching the learning experience for aspiring healthcare professionals.

Discussion

The responses of the participants in this study illuminated the positive aspects and challenges encountered by student midwives during their clinical training in both hospital and community settings. The exploration of their experiences through thematic analysis revealed valuable insights into the complexities of their education and training, shedding light on the multifaceted nature of their journey towards

becoming proficient midwives.

Opportunities that student midwives encountered in their clinical practicum were evident across hospital and community settings, each contributing to their growth and development in distinct ways (Fig. 1). The consistent theme of a supportive learning environment occurred prominently, emphasizing the value of guidance and mentorship from experienced practitioners. This sentiment echoes existing research that underscores the pivotal role of experienced professionals in facilitating knowledge transfer and skill development among novice learners (Tobiano et al., 2019). The willingness of nurses, senior midwives, and experienced practitioners in both hospital and community settings to share their expertise, explain procedures, and patiently address questions fosters an atmosphere of collaboration and mutual respect. The act of imparting knowledge not only accelerates the learning curve of student midwives but also cultivates a culture of continuous learning and mentorship (Neary, 2000).

Moreover, the participants appreciated the diverse clinical experiences offered in both settings, recognizing the value of exposure to a wide range of cases and scenarios. This diversity enhances their clinical competency by honing their abilities to manage complex situations and develop a holistic understanding of healthcare delivery (McNair et al., 2016). Although bounded to the limitations of midwifery practice, the spectrum of cases encountered in hospitals by student midwives, including high-risk pregnancies and urgent situations, enables them to cultivate critical thinking and problem-solving skills—a hallmark of a well-prepared healthcare practitioner. Similarly, community settings provide them with insights into home-based care and personalized support, preparing them to address the distinct needs of patients within their social and cultural contexts. These diverse clinical experiences align with research emphasizing the importance of varied clinical exposure in shaping adaptable and proficient healthcare professionals (Patterson et al., 2019).

Furthermore, the opportunity for hands-on learning and skill development was consistently highlighted by participants. The practical skills acquired during their clinical practicum are integral to their future roles as midwives, enabling them to perform procedures and interventions confidently. This aligns with research emphasizing the significance of practical skill acquisition and its correlation with improved patient outcomes (Birks et al., 2017). The combination of hands-on learning and theoretical knowledge equips student midwives with the competencies required to provide safe and effective patient care. The distinctiveness of skill development in community settings, which

emphasizes holistic care and relationship-building, enriches their understanding of patient-centered care and the importance of interpersonal relationships in healthcare delivery.

The participants also recognized the educational value inherent in exposure to real-world challenges, which foster the development of essential cognitive skills and resilience. The acute decision-making demands of hospital settings contribute to the enhancement of critical thinking and problem-solving abilities. Similarly, community settings cultivate resourcefulness and adaptability, equipping student midwives with the capacity to navigate constraints and innovate solutions—an attribute crucial in contemporary healthcare settings. This aligns with existing research emphasizing the value of experiential learning and its role in fostering practical skills, adaptability, and cognitive competencies (Errasti-Ibarrondo et al., 2019).

In addition to the positive aspects, the participants' experiences also unveiled a spectrum of challenges (Fig. 2), contributing to a comprehensive understanding of the intricacies within hospital and community settings. These challenges provided valuable insights into the multifaceted nature of clinical education and training for student midwives. The occurrence of these challenges highlighted the nuanced dynamics present in both healthcare environments, shedding light on areas that require attention and improvement.

A significant challenge generated during the study was the hierarchical structure prevalent within hospital settings, which occasionally impeded effective communication and access to guidance. This aspect highlighted the pervasive influence of power dynamics within healthcare institutions, potentially affecting the learning journey of student midwives. This observation aligns with existing research in healthcare education, exemplified by the work of Eccott et al. (2012), emphasizing the critical role of establishing open communication channels to enhance learning outcomes (Funa et al., 2023). However, it's crucial to acknowledge that the resolution of hierarchical barriers within midwifery is multifaceted and may require more nuanced strategies beyond fostering open communication alone. While fostering open communication channels is undoubtedly a crucial step, addressing deeper-rooted issues within the hierarchical structure demands a comprehensive approach, possibly involving changes in institutional policies, professional training programs, and organizational culture.

The distinct scope of practice and responsibilities between midwives and nurses can exacerbate confusion, particularly if the delineation between their roles is not clearly communicated. Student midwives may struggle to understand where their responsibilities begin and end in

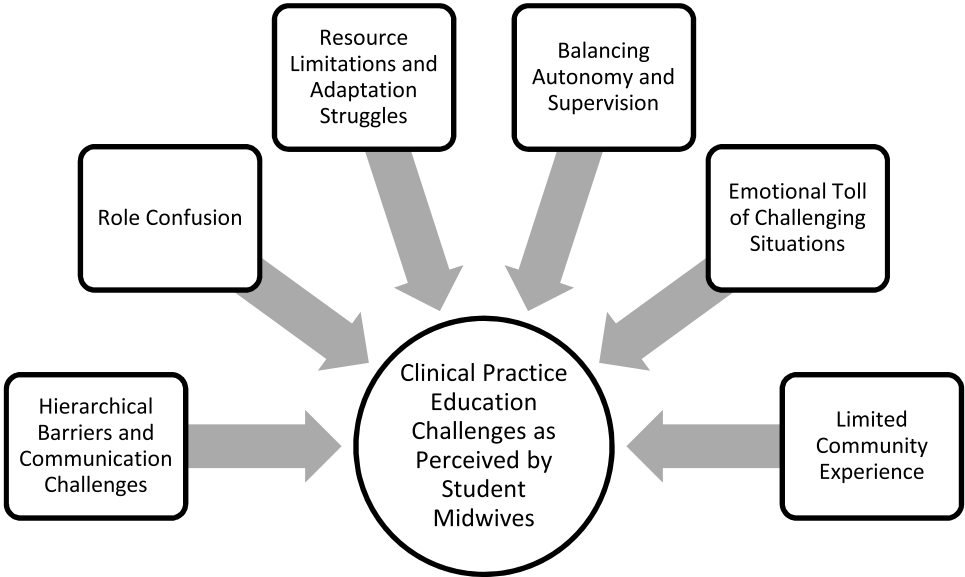


Fig. 2. Clinical practice education challenges as perceived by student midwives.

relation to those of nurses, leading to inefficiencies in patient care delivery and potential conflicts within the healthcare team (Jayathilake et al., 2016). Moreover, the hierarchical structure prevalent in many healthcare settings may further perpetuate role confusion, as student midwives may feel hesitant to assert themselves or seek clarification from more experienced nurses (Jayathilake et al., 2016). To address these challenges, it is essential for educational institutions to provide targeted training and support to student midwives, emphasizing the unique role of midwifery within the healthcare team and promoting effective communication and collaboration between midwives and nurses. By fostering a clearer understanding of roles and promoting mutual respect between midwives and nurses, student midwives can navigate their practicum with confidence and develop the skills necessary for successful integration into the healthcare workforce as competent and collaborative practitioners.

The participants' encounters with resource limitations within community settings illuminated a significant challenge that student midwives faced. The scarcity of resources required innovative thinking and adaptive strategies. Navigating such constraints highlighted the importance of cultivating resourcefulness—a skill that holds relevance beyond education and can significantly impact healthcare delivery even in resource-limited environments (Palacholla et al., 2019). The challenge extends beyond just the lack of supplies in both hospital and community settings; it also encompasses the financial constraints faced by students, such as transportation costs. This finding underscores the need to equip student midwives with the ability to think creatively and manage limited resources effectively. Such skills are essential for maintaining quality care under diverse and challenging circumstances.

The challenge of balancing autonomy and supervision that showcased the intricate process student midwives navigate as they transition from novice learners to autonomous practitioners. Striking the right equilibrium between gaining practical experience and ensuring patient safety is a dynamic that requires careful consideration. This theme resonates with the findings of Happel et al. (2018), which underscored the gradual evolution of healthcare students' responsibilities as they mature into competent practitioners. Acknowledging this balance and offering appropriate guidance can foster a smoother transition and bolster student midwives' confidence in their abilities.

Moreover, the emotional toll of challenging situations was identified as a significant theme, bringing to light the emotional labor inherent in healthcare professions. This finding emphasizes the importance of supporting student midwives in developing effective coping mechanisms to manage the emotional challenges that arise from patient care. Similar insights have been underscored in previous research, such as the study by Lee and Chelladurai (2018), which advocated for providing students with tools to navigate the emotional demands of healthcare roles. Acknowledging and addressing the emotional aspects of clinical training can contribute to the overall well-being and resilience of future midwives.

Lastly, the limited community experience presents a notable challenge for student midwives, characterized by repetitive and limited exposure to diverse clinical scenarios in community settings. This lack of variety may hinder the development of critical thinking skills and may lead to a decrease in student motivation and engagement (Funa et al., 2021; Funa, 2024). To address this issue, educational institutions may consider diversifying clinical placements, incorporating simulation-based learning, and fostering reflective practice to ensure that student midwives receive comprehensive training that prepares them for the complexities of their future roles.

Limitations of the study

While this study offers valuable insights into student midwives' experiences, its limitations should be noted. These include sampling bias due to homogeneity sampling and a small sample size. The researcher's dual role as a Clinical Instructor may introduce biases, and the narrow

focus on challenges during clinical education overlooks other influential factors. Additionally, subjective thematic analysis may affect credibility, while language and cultural barriers and ethical considerations add complexity. Recognizing and addressing these limitations in future research can enhance understanding in midwifery education.

Conclusion and recommendations

The findings of this study reveal a comprehensive understanding of the multifaceted experiences of student midwives during their clinical practicum in both hospital and community settings. Through thematic analysis, the researcher gained insight into the positive aspects and challenges encountered by student midwives, shedding light on the complexities inherent in their educational journey. The study illuminated the value of a supportive learning environment, diverse clinical experiences, hands-on learning and skills development, as well as exposure to real-world challenges in both hospital and community settings.

The positive experiences expressed by participants highlight the significance of mentorship and guidance from experienced practitioners, as well as the opportunity to encounter a wide array of clinical scenarios. The hands-on learning and skill development afforded to student midwives are crucial in equipping them with the competencies necessary for effective clinical practice. Furthermore, the exposure to real-world challenges cultivates critical thinking, problem-solving skills, and adaptability—an essential attribute in the ever-evolving landscape of healthcare.

However, the challenges identified within the study underscore the need for addressing certain barriers to effective clinical training. The hierarchical structure and communication challenges, the role confusion, the resource limitations including financial constraints, the balance between autonomy and supervision, the emotional toll of challenging situations, and limited community experience all require careful consideration. These challenges, while indicative of the complexities within healthcare education, present opportunities for improvement and growth.

Based on the study findings, it is advised that educational institutions and healthcare settings proactively enhance student midwives' clinical training. Firstly, foster open communication between student midwives, experienced practitioners, and supervisors to overcome hierarchical barriers and facilitate effective knowledge sharing. Incorporate resource management education into the curriculum to equip student midwives with strategies for navigating limitations and optimizing resources. Develop guidelines for balancing autonomy and supervision to ensure a smooth transition. Additionally, integrate emotional resilience training to help student midwives manage the emotional demands of patient care. Provide enriched exposure to diverse cases in community settings for a broader range of clinical experiences. Promote interprofessional education to foster collaboration among healthcare disciplines, enhancing patient care and teamwork. Prioritize continuous professional development for both student midwives and experienced practitioners, cultivating a culture of lifelong learning. Future studies may further explore the dynamics of interprofessional collaboration and assess the long-term impact of the recommended interventions on the professional development and performance of student midwives.

CRediT authorship contribution statement

Aaron A. Funa: Writing – review & editing, Writing – original draft, Methodology, Investigation, Formal analysis, Data curation, Conceptualization.

Declaration of competing interest

I have no conflict of interest to disclose.

Acknowledgement

This research has received no external funding.

Supplementary materials

Supplementary material associated with this article can be found, in the online version, at [doi:10.1016/j.midw.2024.104186](https://doi.org/10.1016/j.midw.2024.104186).

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